

2 SCHEDULE OF ACTIVITIES

An overview of the schedule of major assessments in this study are provided in [Table 2-1](#) (Screening), [Table 2-2](#) (On Treatment), and [Table 2-3](#) (Long-term Follow-up).

Table 2-1: Screening Procedural Outline (CA045020)

Procedure	Screening Visit ^a	Notes
Eligibility Assessments		
Informed Consent	X	Register in Interactive Response Technology (IRT) system to obtain participant number. The participant or legally authorized representative should sign the informed consent prior to any study-related assessment is performed. Study allows for re-enrollment of a participant that is a pre-treatment failure and was not previously treated. If re-enrolled, the participant must be re-consented and assigned a new participant number from IRT.
Inclusion/Exclusion Criteria	X	Must be confirmed prior to treatment. See Sections 6.1 and 6.2 .
Medical History	X	All relevant medical history including smoking history, and prior anti-cancer therapies.
Safety Assessments		
Physical Examination	X	Include vital signs (blood pressure, heart rate, and temperature) plus height, weight, and pulse oximetry within 14 days prior to treatment.
Performance Status	X	Within 14 days prior to treatment, collect Lansky play score for ≤ 16 years or Karnofsky performance score for > 16 years of age (Appendix 7).
Prior and Concomitant Medication Use	X	Must be collected within 14 days prior to treatment. Vaccine use within 30 days prior to treatment must be collected.
Serious Adverse Events (SAE) Assessment	X	SAEs collected and reported from time of consent. All AEs (SAEs or non-serious AEs) related to severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2) infection collected from time of consent.
Laboratory Tests		
Clinical Laboratory Testing	X	All laboratory assessments to be performed within 14 days prior to treatment, except for viral testing, which, if clinically indicated, is to be completed within 28 days prior to treatment (see Section 9.4.6 for a list of laboratory tests to conduct).
Electrocardiogram	X	Within 14 days prior to treatment (see Section 9.4.3).

Table 2-1: Screening Procedural Outline (CA045020)

Procedure	Screening Visit ^a	Notes
Echocardiogram or Multigated Acquisition Scan (MUGA)	X	Left ventricular ejection fraction > 45% within 60 days is required prior to treatment assignment. The investigator should further evaluate participants with other significant abnormalities on echocardiogram/MUGA. Decision regarding treatment should be based on investigator's best clinical judgment.
Pregnancy Test	X	Women of childbearing potential only. Serum or urine pregnancy test (minimum sensitivity 25 IU/L or equivalent units of human chorionic gonadotropin) to be done at screening visit and within 24 hours of first dose of study therapy.
Tumor Assessment		
Body Imaging	X	- Imaging to be performed within 28 days prior to treatment. See Section 9.1.1 for further details. - Not applicable to participants with leukemia. - Skull base to mid-thigh [¹⁸F]fluorodeoxyglucose (FDG) positron emission tomography-computed tomography (PET-CT) or PET-magnetic resonance imaging (PET-MRI); additional anatomy to include all sites of disease) for FDG-avid non-Hodgkin lymphoma. - Contrast-enhanced CT of the chest, contrast-enhanced CT/magnetic resonance imaging (MRI) of the abdomen, pelvis, and all other known and/or suspected sites of disease. - Participants with metaiodobenzylguanidine (MIBG)-avid neuroblastoma must also undergo MIBG scintigraphy in addition to CT/MRI as above.
Brain Imaging	X	MRI of the brain without and with contrast is required for participants at baseline in participants with a history of brain metastasis or symptoms. Except for glioma, medulloblastoma, ependymoma, and other central nervous system tumors in which an MRI is required, CT of the brain (without and with contrast) can be performed if MRI is contraindicated. Brain imaging must be done within 28 days prior to treatment. See Section 9.1.1 for further details.
Cerebrospinal Fluid (CSF) - Solid Tumors	X	CSF cytopathology should be obtained in any participant with a suspicion or high likelihood of CSF dissemination up to 7 days prior to treatment. This sample can be used as the Cycle 1 Day 1 sample. If initially positive, CSF cytopathology should be followed as part of response assessment. See Section 9.1 . [REDACTED]

Table 2-1: Screening Procedural Outline (CA045020)

Procedure	Screening Visit ^a	Notes
Bone Marrow and Buccal Swab - Solid Tumors	X	For participants with suspected bone marrow disease, bone marrow aspirate obtained where clinically indicated should be submitted along with buccal swab up to 28 days prior to treatment. If initially positive, bone marrow should be evaluated as part of response assessment. See Section 9.1.
CSF - Leukemia	X	Screening sample collected up to 7 days prior to treatment initiation can be used as Cycle 1 Day 1 pretreatment sample. See Section 9.1.2.5.
Bone Marrow and Buccal Swab - Leukemia	X	Bone marrow aspirate collected at screening and submitted along with buccal swab up to 28 days prior to treatment initiation can be used as Cycle 1 Day 1 pre-treatment sample. See Section 9.1.2.5.

Abbreviations: CSF = cerebrospinal fluid; CT = computed tomography; FDG = [¹⁸F]fluorodeoxyglucose; IRT = Interactive Response Technology; MIBG = metaiodobenzylguanidine; MRI = magnetic resonance imaging; MUGA = multigated acquisition; PET = positron emission tomography; SAE = serious adverse event; SARS-CoV-2 = severe acute respiratory syndrome coronavirus 2.

^a Some of the assessments referred to in this section may not be captured as data in the electronic case report form. They are intended to be used as safety monitoring by the treating physician. Additional testing or assessments may be performed as clinically necessary or where required by institutional or local regulations.

Table 2-2: On-treatment Procedural Outline (CA045020)

Procedure	Cycle 1 Only ^{a,b} Cycle = 3 wks				Cycle 2 and Beyond ^{a,b,c} Each cycle = 3 wks		Notes
	Day 1	Day 3 (± 1 day) ^d	Day 5 (± 1 day)	Day 8 (- 1 day)	Day 1	Day 3-5	
Safety Assessments							
Targeted Physical Examination, Vital Signs, and Performance Status	X			X*	X		Vital signs (blood pressure, heart rate, and temperature), weight, Lansky play score for ≤ 16 years or Karnofsky performance score for > 16 years of age (Appendix 7). Monitor and record vital signs at predose and within 30 minutes after administration of nivolumab. *At Day 8 visits collect vital signs only.
12-lead Electrocardiogram (ECG)	X	X	X	X	X	X (Cycle 5 only)	Pre-dose Day 1 of all cycles Pre-PK blood draw on Cycle 1, Days 3,5, and 8 Pre-PK blood draw on Cycle 5, Days 3 and 5 ECG should be performed within 30 minutes prior to PK blood draw. Participant should rest for 5 minutes prior to each ECG
Adverse Events Assessment (including Serious Adverse Events)		Continuously					Adverse events (AEs) will be graded according to the National Cancer Institute Common Terminology Criteria for AEs (NCI-CTCAE) version 5. Record at each visit. All AEs (SAEs or non-serious AEs), including those associated with SARS-CoV-2 infection, must be collected continuously throughout the treatment period and for a minimum of 150 days after last dose of study treatment (Appendix 3).

Table 2-2: On-treatment Procedural Outline (CA045020)

Procedure	Cycle 1 Only ^{a,b} Cycle = 3 wks				Cycle 2 and Beyond ^{a,b,c} Each cycle = 3 wks		Notes
	Day 1	Day 3 (± 1 day) ^d	Day 5 (± 1 day)	Day 8 (- 1 day)	Day 1	Day 3-5	
Concomitant Medication Use		Continuously					Record at each visit.
Laboratory Tests							
Clinical Laboratory Testing	X			X	X		Hematology and chemistry assessments scheduled for the day of study drugs dosing must be available and assessed before dosing. Hematology and chemistry assessments can be drawn within 72 hours prior to treatment, though renal function (ie, serum creatinine) must be assessed within 24 hours prior to dosing with bempegaldesleukin or as soon as locally feasible. Refer to Section 9.4.6 for a list of laboratory tests to conduct.
Pregnancy Test	X				X		Serum or urine pregnancy test (minimum sensitivity units 25 IU/L or equivalent units of human chorionic gonadotropin) is required within 24 hours prior to treatment in women of childbearing potential.

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	Day 1	Day 3 (± 1 day) ^d	Day 5 (± 1 day)	Day 8 (- 1 day)	Day 1	Day 3-5	
Efficacy Assessments							
Body Imaging		- Imaging should occur every 9 weeks (± 7 days) from start of treatment for the first 54 weeks, then every 12 weeks (± 7 days) until disease progression or treatment discontinuation (including treatment beyond progression), whichever occurs later. Tumor assessment schedule should be maintained regardless of dose delays. Use same imaging method as was used at screening. - Not applicable to participants with leukemia. - Skull base to mid-thigh [¹⁸F]fluorodeoxyglucose (FDG) positron emission tomography-computed tomography (PET-CT) or PET-magnetic resonance imaging (PET-MRI); additional anatomy to include all sites of disease) for FDG-avid non-Hodgkin lymphoma. - Contrast-enhanced CT of the chest, contrast-enhanced CT/magnetic resonance imaging (MRI) of the abdomen, pelvis, and all other known and/or suspected sites of disease. - Participants with metaiodobenzylguanidine (MIBG)-avid neuroblastoma must also undergo MIBG scintigraphy in addition to CT/MRI as above.				All study treatment decisions will be based on the investigator's assessment of tumor images. See Section 9.1.1 for additional details and to Section 8.1.2 for tumor assessment associated with treatment beyond progression.	

Table 2-2: On-treatment Procedural Outline (CA045020)

Procedure	Cycle 1 Only ^{a,b} Cycle = 3 wks				Cycle 2 and Beyond ^{a,b,c} Each cycle = 3 wks		Notes
	Day 1	Day 3 (± 1 day) ^d	Day 5 (± 1 day)	Day 8 (- 1 day)	Day 1	Day 3-5	
Brain Imaging		Participants in brain tumor-related cohorts and those with a history of brain metastasis or symptoms should undergo MRI of the brain without and with contrast every 9 weeks (± 7 days) from start of treatment up to 54 weeks, and then approximately every 12 weeks (± 7 days), or sooner if clinically indicated. Tumor assessment schedule should be maintained regardless of dose delays. Cerebrospinal fluid (CSF), if clinically indicated, should be followed as per Response Assessment in Pediatric Neuro-Oncology guidelines.					Except for glioma, medulloblastoma, ependymoma and other central nervous system tumors in which an MRI is required, CT of the brain (without and with contrast) can be performed if MRI is contraindicated. See Section 9.1.1 for further details.
CSF - Solid tumors		Performed at time of imaging assessments for disease response as clinically indicated.					CSF cytopathology should be obtained in any participant with a previously positive CSF as part of response assessment (see Section 9.1). [REDACTED]
Bone Marrow-Solid tumors		Performed at time of imaging assessments for disease response as clinically indicated.					Bone marrow biopsy/aspirate should be obtained in any participant with a previously positive bone marrow aspirate as part of response assessment (see Section 9.1). [REDACTED]
CSF - Leukemia		Performed as clinically indicated.					CSF cytopathology should be obtained in any participant with a previously positive CSF as part of response assessment (see Section 9.1.2.5). [REDACTED]

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Procedure	Cycle 1 Only ^{a,b} Cycle = 3 wks				Cycle 2 and Beyond ^{a,b,c} Each cycle = 3 wks		Notes
	Day 1	Day 3 (± 1 day) ^d	Day 5 (± 1 day)	Day 8 (- 1 day)	Day 1	Day 3-5	
Bone Marrow - Leukemia		Performed as clinically indicated.					Bone marrow biopsy/aspirate should be obtained in any participant with a previously positive bone marrow aspirate as part of response assessment.
Pharmacokinetic (PK)/Immunogenicity Assessments ^e							
Bempegaldesleukin PK Plasma Samples		See Section 9.5.1 for further details.					
Nivolumab PK Serum Samples		See Section 9.5.1 for further details.					
Immunogenicity Samples		See Section 9.5.1 for further details.					

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Procedure	Cycle 1 Only ^{a,b} Cycle = 3 wks				Cycle 2 and Beyond ^{a,b,c} Each cycle = 3 wks		Notes
	Day 1	Day 3 (± 1 day) ^d	Day 5 (± 1 day)	Day 8 (- 1 day)	Day 1	Day 3-5	
Health Outcomes							
Patient-reported Outcomes (PRO) Version of the CTCAE		See Notes					Questionnaire to be administered electronically in Part B only, on a weekly basis during the first 3 cycles of treatment, every cycle from Cycles 4 through 8, and then every other cycle (starting at Cycle 10) while on treatment until end of treatment. PRO-CTCAE™ for adults ≥18 years. Pediatric (Ped-PRO-CTCAE) for children ≥7 to <18 years. Ped-PRO-CTCAE [Caregiver] for children <7 years.
Study Drug							
Determine Dosing Schema	X						See Figure 5.1-1 , Figure 5.1-2 and Figure 5.1-3 .
Administer Intravenous Fluids	X				X		May be withheld if deemed by the investigator to be in the best interest of the participant. Refer to Section 7.1.1.1 for additional information.
Administer Bempegaldesleukin + Nivolumab (Parts A and B)	X				X		

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Procedure	Cycle 1 Only ^{a,b} Cycle = 3 wks				Cycle 2 and Beyond ^{a,b,c} Each cycle = 3 wks		Notes
	Day 1	Day 3 (± 1 day) ^d	Day 5 (± 1 day)	Day 8 (- 1 day)	Day 1	Day 3-5	
Review Hydration Guidelines with Participants	X				X		See Section 7.1.1.1 .
Oral Hydration Follow-up			X (Day 3-5)			X	For the first 2 doses of bempegaldesleukin, between Days 3 and 5 following infusion, site personnel must contact the participant (by telephone or clinic visit) to remind the participant of the oral hydration guidelines, to assess for any symptomatology and compliance with the guidelines, and document the results of the discussion. For subsequent administrations of bempegaldesleukin, the oral hydration follow-up should be conducted as clinically indicated for participants receiving bempegaldesleukin. See Section 7.1.1.1 for details.

Abbreviations: AE = adverse event; CSF = cerebrospinal fluid; CT = computed tomography; CTCAE = Common Terminology Criteria for Adverse Events version 5; ECG = electrocardiogram; FDG = [¹⁸F]fluorodeoxyglucose; ██████████ MIBG = metaiodobenzylguanidine; MRI = magnetic resonance imaging; NCI = National Cancer Institute; Ped = pediatric; PET = positron emission tomography; PK = pharmacokinetic; PRO = patient-reported outcome; SARS-CoV-2 = severe acute respiratory syndrome coronavirus 2.

^a Some of the assessments referred to in this section may not be captured as data in the electronic case report form. They are intended to be used as safety monitoring by the treating physician. Additional testing or assessments may be performed as clinically necessary or where required by institutional or local regulations.

^b If a dose is delayed, the procedures scheduled for that same time point, with the exception of the body/brain imaging, should also be delayed to coincide with when the time point's dosing actually occurs.

^c For Cycle 5 only, participants will return for additional sample collection at Days 3 and 5. Refer to [Section 9.5.1 \(Table 9.5.1-1\)](#) for all details, including allowed windows.

- ^d Samples can be collected $\pm$ 3 hours. If participants are dosed on a Thursday, Day 3 samples may be collected on Day 2. If dosed on a Friday, Day 3 sample can be collected on Day 4.
- ^e For participants who require blood draw volume modifications, blood collection volumes will be reduced per guidelines outlined in the laboratory manual. Omitted blood collection will likely be for [REDACTED] pharmacokinetic assessments, since all required safety assessments must be performed.

Table 2-3: Long-term Follow-up Period (CA045020)

Procedure	Safety Follow-up Visit 1 ^a	Safety Follow-up Visit 2 ^a	Safety Follow-up Visit 3 ^a	Survival Follow-up Every 3 Months (± 14 Days) ^b	Notes ^c
Safety Assessments					
Vital Signs	X	X	X		Blood pressure, heart rate, temperature.
Performance Status	X		X		Lansky play score for ≤ 16 years or Karnofsky performance score for > 16 years of age (Appendix 7).
12-lead ECG	X				Participant should rest for 5 minutes prior to each ECG.
Adverse Events and Serious Adverse Events (SAE) Assessment	X	X	X	See Notes	All SAEs and non-serious AEs should be collected continuously during the treatment period and for a minimum of 150 days following discontinuation of study treatment. Participants will be followed for all SAEs, non-serious AEs of special interest (as defined in Section 9.2.8) and all AEs (SAEs and non-serious AEs) associated with confirmed or suspected SARS-CoV-2 infection until resolution, the condition stabilizes, the event is otherwise explained, the event is deemed irreversible, the participant is lost to follow-up (as defined in Section 8.3), or for suspected cases, until SARS-CoV-2 infection is ruled-out.
Subsequent Medications	X	X	X		
Review of Subsequent Anti-cancer Therapy	X	X	X	X	Additional subsequent cancer therapy details such as regimen, setting of the regimen, line of therapy, start and end date of each regimen, best response to the regimen, and date of progression to subsequent anti-cancer therapies will be collected.

Table 2-3: Long-term Follow-up Period (CA045020)

Procedure	Safety Follow-up Visit 1 ^a	Safety Follow-up Visit 2 ^a	Safety Follow-up Visit 3 ^a	Survival Follow-up Every 3 Months (± 14 Days) ^b	Notes ^c
Laboratory Tests					
Laboratory Tests (includes blood and urine samples)	X	If toxicities are present.	If toxicities are present		See Section 9.4.6 for the list of laboratory tests.
Pregnancy Test	X	X	X		Women of childbearing potential only. Serum or urine pregnancy test (minimum sensitivity equivalent units 25 IU/L or equivalent units of human chorionic gonadotropin) is required.

Table 2-3: Long-term Follow-up Period (CA045020)

Procedure	Safety Follow-up Visit 1 ^a	Safety Follow-up Visit 2 ^a	Safety Follow-up Visit 3 ^a	Survival Follow-up Every 3 Months (± 14 Days) ^b	Notes ^c
Efficacy Assessments					
Body Imaging	- Imaging should occur every 9 weeks (± 7 days) from start of treatment for the first 54 weeks, then every 12 weeks (± 7 days) until disease progression (including treatment beyond progression) or end of study, whichever occurs later. Use same imaging method as was used at screening. After completion of study therapy for participants without progression, results of additional imaging (including bone marrow and CSF, if applicable) are requested for the duration of the study. - Not applicable to participants with leukemia. - Skull base to mid-thigh [¹⁸F]fluorodeoxyglucose (FDG) positron emission tomography-computed tomography (PET-CT) or PET-magnetic resonance imaging (PET-MRI); additional anatomy to include all sites of disease for FDG-avid non-Hodgkin lymphoma. - Contrast-enhanced CT of the chest, contrast-enhanced CT/magnetic resonance imaging (MRI) of the abdomen, pelvis, and all other known and/or suspected sites of disease. - Participants with metaiodobenzylguanidine (MIBG)-avid neuroblastoma must also undergo MIBG scintigraphy in addition to CT/MRI as above.				See Section 9.1.1 for further details.
Brain Imaging	Participants in brain tumor-related cohorts and those with a history of brain metastasis or symptoms should undergo MRI of the brain without and with contrast every 9 weeks (± 7 days) from start of treatment up to 54 weeks, and then approximately every 12 weeks (± 7 days), or sooner if clinically indicated until disease progression (including treatment beyond progression) or end of study, whichever occurs later. Cerebrospinal fluid (CSF), if clinically indicated, should be followed as per Response Assessment in Pediatric Neuro-Oncology guidelines.				Except for glioma, medulloblastoma, ependymoma, and other central nervous system tumors in which an MRI is required, CT of the brain without and with contrast can be performed if MRI is contraindicated. See Section 9.1.1 for further details.

Table 2-3: Long-term Follow-up Period (CA045020)

Procedure	Safety Follow-up Visit 1 ^a	Safety Follow-up Visit 2 ^a	Safety Follow-up Visit 3 ^a	Survival Follow-up Every 3 Months (± 14 Days) ^b	Notes ^c
CSF - solid tumors and leukemia	As clinically indicated.				CSF cytopathology should be obtained in any participant with a previously positive CSF as part of response assessment. See Section 9.1
Bone marrow aspirate - solid tumors and leukemia	As clinically indicated				Bone marrow aspirate should be obtained in any participant with a previously positive bone marrow aspirate as part of response assessment. See Section 9.1 .
Survival Status	X	X	X	X	During safety follow-up and every 3 months (clinic visit or by telephone) thereafter and during survival phase. Include documentation of subsequent chemotherapy. See Section 5.1.4 .
Pharmacokinetics (PK)/Immunogenicity Assessment					
PK Samples	See Section 9.5.1 for further details.				
Immunogenicity Samples	See Section 9.5.1 for further details.				

Abbreviations: CSF = cerebrospinal fluid; CT = computed tomography; ECG = electrocardiogram; FDG = [18F]fluorodeoxyglucose; MIBG = metaiodobenzylguanidine; MRI = magnetic resonance imaging; PET = positron emission tomography; SAE = serious adverse event; SARS-CoV-2 = severe acute respiratory syndrome coronavirus 2.

- ^a Participants must be followed for at least 150 days after last dose of study treatment. Follow-up Visit 1 should occur 30 days from the last dose (± 7) days or can be performed on the date of discontinuation if that date is greater than 42 days from last dose. Follow-up Visit 2 occurs approximately 100 days (± 7 days) from last dose of study drug. Follow-up Visit 3 occurs approximately 150 days (± 7 days) from last dose of study drug. All 3 follow-up visits should be conducted in person.
- ^b Survival follow-up visits to occur every 3 months subsequent to Follow-up Visit 3. Survival visits may be conducted in person or by telephone. Bristol-Myers Squibb may request that survival data be collected on all treated participants outside of the 3 month specified window. At the time of this request, each participant will be contacted to determine their survival status unless the participant has withdrawn consent for all contact.
- ^c Some of the assessments referred to in this section may not be captured as data in the electronic case report form. They are intended to be used as safety monitoring by the treating physician. Additional testing or assessments may be performed as clinically necessary or where required by institutional or local regulations.